QUAERITE

Seeking Cures.
Saving Sight.

Advancing next-generation biologics for vision-threatening retinal diseases

PHASE I-READY | 4 LEAD PROGRAMS | OCUHLE-X® PLATFORM

Building the next leader in retinal biologics.

A differentiated, science-led pipeline of long-acting multi-specific biologics targeting the largest unmet needs in wAMD, GA, DME and DR.

01

Differentiated pipeline

Four lead programs across the highest-value retinal indications, designed to outperform standard of care.

02

Proprietary platform

OcuHLE-X® extends ocular half-life; modular multi-specific design enables best-in-class molecules.

03

BD-ready assets

Strong preclinical packages, IP filings in place, and asset-by-asset partnering optionality.

04

World-class team

Founded by Tsinghua leadership, ex-BMS and global biotech operators, with leading retinal-disease KOLs.

4

Lead clinical-track programs

7+

Indications addressed

$15B+

Anti-VEGF + complement market

OcuHLE-X®

Proprietary PK platform

/ THE VISION

Vision loss is one of medicine's last great unmet needs.

And we refuse to accept it. Quaerite was founded to engineer the molecules that finally bend the curve.

Behind every statistic is someone losing the world.

Retinal disease is the leading cause of irreversible blindness in the developed world. Despite a generation of anti-VEGF therapy, millions still progress to vision loss — because today's medicines fall short in durability, efficacy and mechanism.

200M+	~50%	5M+	$15B+
People worldwide live with wAMD and DME	Of anti-VEGF patients fail to achieve full vision gain	Patients affected by geographic atrophy globally	Annual market for retinal biologics — and growing

Sources: WHO Vision 2020 · AAO · GlobalData retinal-disease forecasts

QUAERITE

/kwah-EH-ri-tay/

Latin: "to seek, to search, to inquire."

Our name is our discipline: rigorous inquiry into the biology of sight.

OUR MISSION

To engineer the next generation of ocular biologics — molecules that act longer, reach deeper, and work where today's standard of care falls short.

WHAT WE BUILD

Multi-specific antibodies and engineered fusion proteins that combine validated targets with novel biology — designed from the molecule up for the retina's unique pharmacology.

HOW WE BUILD IT

Insight-driven discovery · proprietary OcuHLE-X® PK platform · biology-led indication selection.

Three differentiators. One thesis.

Why our biologics can outperform the current generation of retinal therapies.

01

Insight-driven discovery

Deep biology mapping of retinal disease pathways drives target selection and epitope design — every molecule starts from a clear mechanistic thesis.

02

Modular multi-specific platform

Plug-and-play antibody architecture combines validated and novel targets to address heterogeneous disease biology with a single intravitreal injection.

03

Proprietary long-acting design

OcuHLE-X® extends ocular half-life and optimizes tissue distribution — pushing dosing intervals beyond what monoclonal antibodies achieve.

/ THE OPPORTUNITY

A market that has outgrown yesterday's medicines.

Anti-VEGF unlocked the retinal biologics market. The next wave of value will go to therapies that solve what anti-VEGF cannot.

QUAERITE

Retinal biologics — a $20B market expanding into new frontiers.

Anti-VEGF set the foundation. Long-acting multi-specifics, complement therapy and DR-prevention build the next tier.

MARKET OPPORTUNITY

TAM
Global retinal Rx
$20B+
by 2030

SAM
Long-acting+multi-spec
~$12B
ex-VEGF biosimilars

SOM
Lead-asset reach
~$4B
QBR202 + QBR207

WHERE GROWTH IS HEADED

SEGMENT	STATUS	GROWTH DRIVER
Anti-VEGF wAMD · DME · DR	Mature	Durability, multi-target
Geographic atrophy	**Emerging**	Efficacy, fewer injections
Diabetic retinopathy	**Underserved**	Earlier intervention
Rare ocular indications	**White space**	Orphan-drug economics

Sources: EvaluatePharma · GlobalData ophthalmology · company estimates · figures rounded.

Anti-VEGF works — until it doesn't.

Real-world durability, compliance and ceiling effects leave billions of efficacy on the table.

TODAY'S STANDARD OF CARE

Lucentis · EYLEA · Vabysmo

- **Frequent dosing** — monthly to bi-monthly intravitreal injections; real-world compliance drops sharply after Year 1.
- **Efficacy ceiling** — 30–50% of patients are suboptimal responders to anti-VEGF monotherapy.
- **Mechanistic gap** — single-pathway blockade misses ANG-2, IL-6, complement and Wnt biology.
- **Vision loss continues** — real-world VA gains are well below clinical-trial outcomes.

WHERE QUAERITE PLAYS

Next-gen · multi-specific · long-acting

- **Extended durability** — OcuHLE-X® engineered for ocular half-life supporting Q3–Q6-month dosing.
- **Higher response rate** — multi-specific design targets biology anti-VEGF leaves untouched.
- **Disease-specific design** — each asset built around the dominant biology of its target indication.
- **Replace or augment SOC** — anti-VEGF replacement (QBR202) and novel-MoA pioneer (QBR207, QBR210).

Four forces converging to reshape retinal therapy.

The next decade in ophthalmology is defined by durability, multi-targeting and novel biology — all areas where Quaerite leads.

01 BIOLOGY

Biology is ready.

Complement, Wnt, IL-6 and ANG-2 pathways have moved from hypothesis to clinical validation — and remain underserved by approved therapies.

02 PATIENTS

Patients demand durability.

Real-world adherence to monthly injections collapses after Year 1; ophthalmologists name durability the #1 unmet need.

03 TECHNOLOGY

Technology is mature.

Multi-specific antibody engineering, PK extension and high-concentration IVT formulation have reached production-scale maturity.

04 CAPITAL

Capital is rotating.

Investors and pharma BD have refocused on retinal disease following SYFOVRE/IZERVAY approvals and Vabysmo's commercial success.

/ THE PIPELINE

Four shots on goal across the biggest retinal markets.

Each Quaerite program is built around a clear unmet need, a validated or novel target, and a differentiated mechanistic edge.

Pipeline for vision-threatening diseases.

Six programs spanning best-in-class, first-in-class and discovery-stage assets — driven by a single technology platform.

PROGRAM	TARGET	2025	2026	2027	INDICATIONS
QBR202	anti-VEGFA/ANG-2 fusion	IND-Enabling	Phase 1 · SAD + MAD	Phase 1 - MAD	wAMD · DME · DR · RVO · mCNV
QBR207	anti-C3/C5 bispecific	PCC	IND-Enabling	Phase 1	Geographic Atrophy
QBR209	VEGF/IL-6/ANG-2 fusion	PCC	IND-Enabling	IND-Enabling	wAMD · DME · DR · UME
QBR210	FZD4/LRP5 Norrin-mimetic	Discovery + PCC	IND-Enabling	Phase 1	wAMD · DME · DR · DMI
QBR211	Undisclosed	Discovery	PCC	IND-Enabling	TED
QBR212	Undisclosed	Discovery	PCC	IND-Enabling	wAMD · DME · DR

STAGE Discovery PCC IND-Enabling Phase 1

PCC: pre-clinical candidate · IND: investigational new drug

One platform. Eight retinal indications. Multiple shots at scale.

Asset × indication coverage — Quaerite addresses the most economically significant ophthalmology markets and clear orphan opportunities.

ASSET	wAMD	DME	DR	RVO	mCNV	GA	TED	UME	DMI
QBR202									
QBR207									
QBR209									
QBR210									
QBR211									
QBR212	●	●	●						

wAMD wet age-related macular degeneration · DME diabetic macular edema · DR diabetic retinopathy · RVO retinal vein occlusion · mCNV myopic CNV · GA geographic atrophy · TED thyroid eye disease ·

/ THE PLATFORM

OcuHLE-X®: the engine behind every Quaerite molecule.

An integrated discovery and engineering platform built specifically for the unique demands of intraocular therapeutics.

OcuHLE-X® — an integrated engine for ocular biologics.

Three complementary capabilities turn novel target biology into molecules with best- or first-in-class potential.

01

Structure & biology- guided discovery

Deep target biology, epitope identification and structure-guided antibody design.

Novel and validated POC targets

Target biology & epitope mapping

Robust bioassays for translation

02

Modular multi-specific architecture

Plug-and-play antibody scaffolds enabling 2×, 3× and biparatopic targeting in one molecule.

Validated multi-specific scaffolds

Modular plug-in of binding modules

Biparatopic & tri-specific designs

03

OcuHLE-X® PK optimization

Proprietary engineering to extend ocular half-life and tailor tissue distribution.

Extended ocular residence time

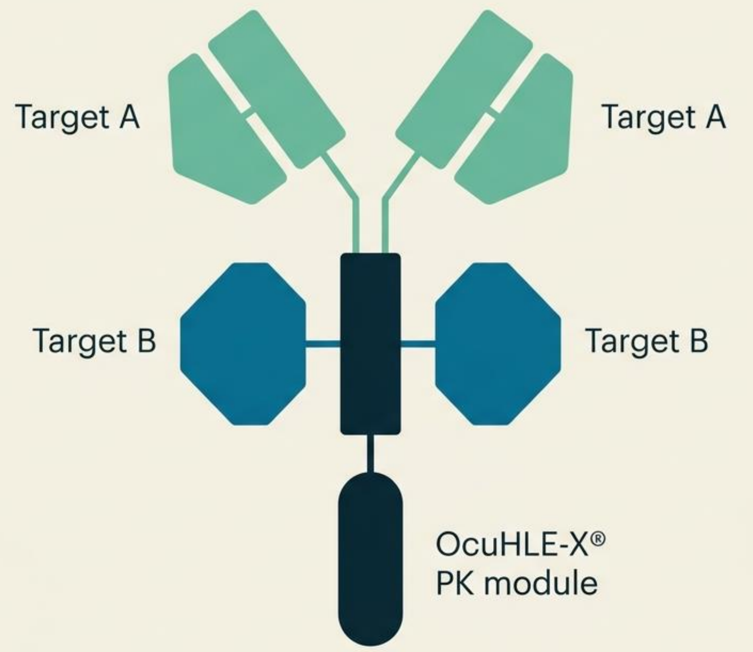
High-concentration formulation

30G IVT injectability

One scaffold, many therapies.

Modular multi-specific architecture lets us swap binding domains to address indication-specific biology without re-engineering the molecule.

MODULAR MULTI-SPECIFIC SCAFFOLD



INSTANTIATED IN THE PIPELINE

QBR202 — anti-VEGFA + anti-ANG-2
Best-in-class durability for wAMD/DME/DR/RVO.

QBR207 — anti-C3 + biparatopic anti-C5
First-in-class complement bispecific for GA.

QBR209 — anti-VEGFA + anti-IL-6 + anti-ANG-2
Triple-targeted for inflammatory retinopathy.

QBR210 — tetravalent FZD4 + LRP5 Norrin-mimetic
Rebuild blood-retina barrier; restore vasculature.

From disease biology to drug candidate.

An insight-led, structure-guided discovery workflow that compounds across the pipeline.

01

Disease biology mapping

Identify the dominant pathways driving each retinal indication via primary tissue, clinical biomarker and external evidence review.

02

Target & epitope selection

Choose targets and epitopes with mechanistic edge and clean clinical translatability.

03

Multi-specific design

Use modular scaffolds to combine validated and novel targets; optimize geometry and affinity.

04

PK optimization

Apply OcuHLE-X® engineering to extend ocular half-life and tune tissue distribution.

OUTPUT › **Best- or first-in-class long-acting biologics, purpose-built for retinal pharmacology.**

QUAERITE

OcuHLE-X®: engineered for retinal exposure and durability.

Significantly extended ocular half-life and improved retinal tissue distribution vs. SOC anti-VEGF therapy.

RETINA/CHOROID EXPOSURE · HVEGF-A MICE

~20× higher exposure at equal IVT dose.

QBR202 **~400**

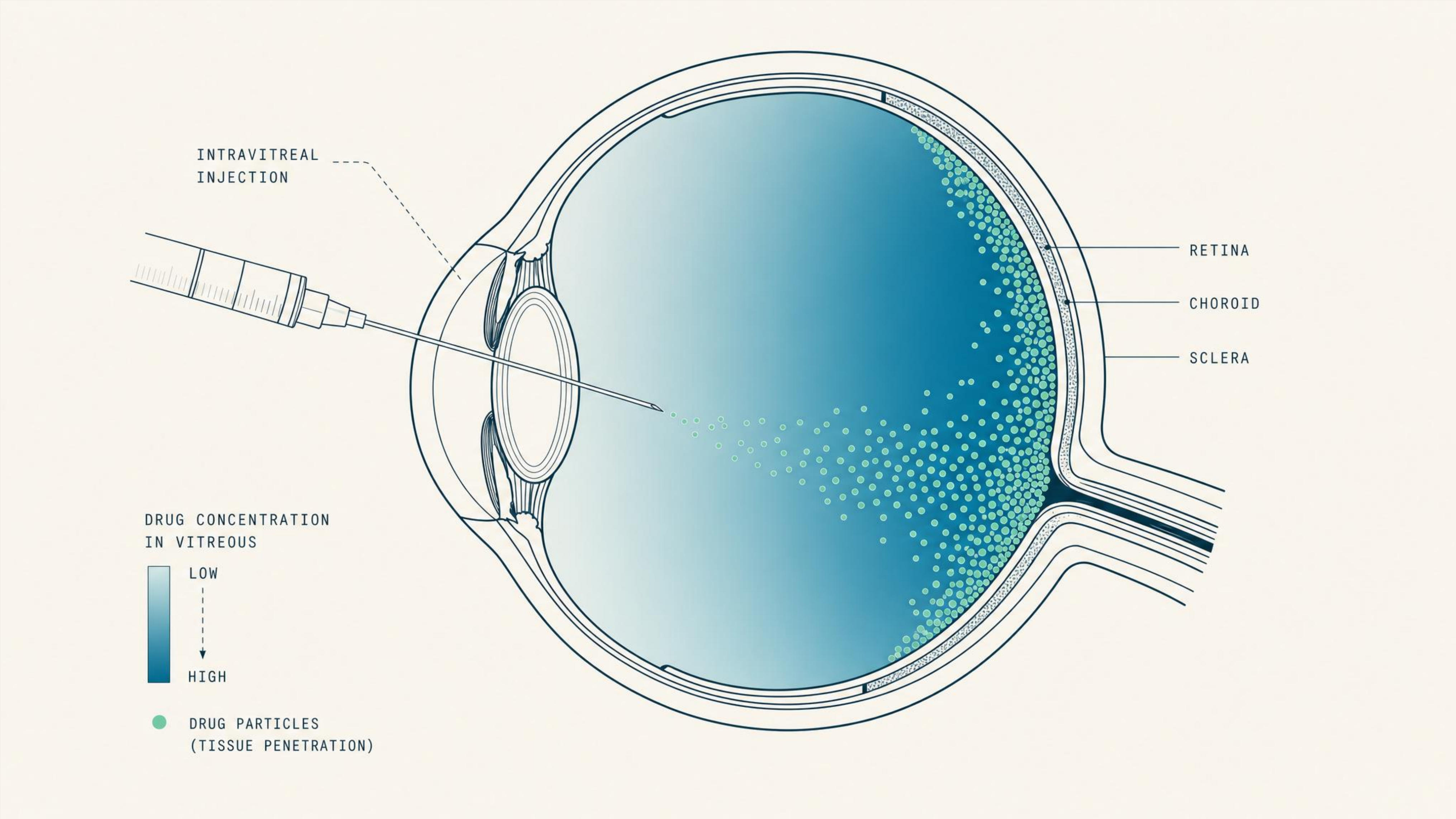
Faricimab **~20**

OCULAR RESIDENCE · RABBIT IVT

Slower vitreous + aqueous clearance vs. Vabysmo™.

TRANSLATIONAL READOUT

Longer ocular residence + deeper tissue penetration = Q4-month dosing potential.



Concentration values illustrative; based on Quaerite in-house preclinical PK studies.

/ LEAD ASSETS

Four molecules. Four shots at category leadership.

Each lead asset is purpose-built for a specific retinal indication — and each has a distinct best- or first-in-class thesis.

202

LEAD ASSET · BEST-IN-CLASS

Next-generation anti-VEGFA / ANG-2 fusion protein.

Engineered for superior pharmacological activity, longer ocular half-life and favorable tissue distribution — positioned as best-in-class therapy for wet AMD and broader anti-VEGF-responsive retinopathies.

Compliance and ceiling effects limit real-world outcomes.

Even with EYLEA and Vabysmo, retinal patients still lose vision. The wAMD/DME market needs a molecule that pushes harder on durability AND mechanism.

TODAY'S PROBLEM

Compliance + ceiling effects

Insufficient compliance — Eylea VIEW-2 shows ~+8 letters BCVA; AURA RWE drops to ~+2 letters at Year 1 due to under-dosing.

Suboptimal responders — 30–50% of anti-VEGF patients achieve <15-letter gain; many have residual fluid and persistent leakage.

Incomplete biology — VEGF-only blockade misses ANG-2, IL-6 and complement driving disease progression.

QBR202 SOLUTION

Long-acting · dual-mechanism

Extended dosing interval — OcuHLE-X®-engineered for projected Q3–Q4-month dosing, superior to Vabysmo's Q16w label.

Dual VEGF + ANG-2 inhibition — synergistic blockade of two angiogenic and inflammatory pathways at the right tissue.

Optimized tissue distribution — higher and more durable exposure in retina/choroid for deeper disease control.

QUAERITE

Dual targeting of VEGF-A and ANG-2 — mechanistic synergy.

Combined inhibition of two converging pathways drives stronger vessel stabilization, reduced leakage and dampened inflammation.

VEGF-A BLOCKADE

Inhibits VEGFR1/VEGFR2 signaling — reduces neovascularization and vascular permeability.

ANG-2 BLOCKADE · TIE-2 SPARING

Inhibits Tie-2 destabilization while preserving ANG-1 binding — restores endothelial stability and dampens inflammation.

SYNERGISTIC ANATOMIC BENEFIT

Improves retinal thickness reduction, fluid resolution and durability beyond anti-VEGF monotherapy.

DUAL-PATHWAY BLOCKADE

Two angiogenic pathways inhibited simultaneously — in one molecule.

SYNERGY THESIS

Blocking only VEGF leaves ANG-2 driving residual disease. QBR202 closes both.

Best-in-class signal across in vitro and in vivo studies.

QBR202 demonstrates superior potency, durable retinal exposure and Faricimab-matching efficacy at a fraction of the dose.

6×

VS. FARICIMAB

Superior in vitro potency

~6× lower IC50 vs. Faricimab on VEGF-A/VEGFR2 ELISA and improved HUVEC proliferation inhibition.

100%

GRADE-4 INHIBITION

Complete NHP CNV inhibition

Complete inhibition of Grade-4 laser-induced CNV lesions in rhesus monkey from Day 14 through end-of-study.

>99%

LEAKAGE REDUCTION

Massive leakage reduction

Significantly reduced retinal thickness and lesion leakage area at Day 28 in NHP CNV model.

0.5–2.5mg

VS. 6 MG FARICIMAB

Dose advantage

0.5/2.5 mg QBR202 matches 6 mg Faricimab in anti-angiogenesis and vascular leakage — supports lower-volume formulation.

All studies conducted with Quaerite in-house manufacturing material · published data on file.

QUAERITE

Best-in-class thesis backed by the science of durability.

Four convergent advantages position QBR202 to outperform Vabysmo and reset the durability ceiling in retinal disease.

01

Extended residence

OcuHLE-X® drives extended ocular half-life and Q4-month dosing potential.

02

In vitro superiority

Higher anti-VEGFA and ANG-2 potency in standardized assays vs. Faricimab.

03

Tissue penetration

Higher exposure in retina/choroid translates to deeper disease-activity control.

04

Durable biomarkers

Preclinical fibrosis-control biomarkers signal long-term visual-outcome benefit.

IMPLICATION › **QBR202 has the profile to capture meaningful share in a $10B+ wAMD/DME/RVO market and to underwrite premium BD economics.**

LEAD ASSET · FIRST-IN-CLASS

First-in-class anti-complement bispecific antibody.

Broad complement-pathway inhibition designed to outperform SYFOVRE and IZERVAY in geographic atrophy — and to translate complement biology into real visual benefit for the first time.

Geographic atrophy: the next big retinal opportunity.

An advanced form of AMD with 5M+ patients globally — and a pathway where complement inhibition is mechanistically proven but clinically underdelivered.

PATHOPHYSIOLOGY

- **Advanced AMD** — gradual, permanent vision loss.
- **Cell loss** — photoreceptor + RPE atrophy in the macula.
- **Drusen origin** — abnormal RPE pigment and drusen deposition.

PREVALENCE

5M+
patients worldwide · ~20% of AMD cases

3–5%
of adults over 75 develop GA

18.5M
projected global cases by 2040

PATIENT IMPACT

2.5 yrs
average to reach the fovea

67%
lose driving ability after 1.6 yrs

−10 letters
half of patients at 2 yrs

Source: Kalpana Rajanala et al., Front. Ophthalmol. 2023 · WHO Vision 2020.

SYFOVRE & IZERVAY opened the market — but didn't close it.

Two FDA-approved C3/C5 inhibitors validate the complement thesis — yet leave meaningful efficacy, safety and convenience gaps.

APELLIS

SYFOVRE

2 peptides → 40 kDa PEG

22%
GA-rate reduction · OAKS, monthly

12%
Wet AMD safety signal

Dosing: every 25–60 days, IVT injection

ASTELLAS / IVERIC BIO

IZERVAY

39-mer RNA → 40 kDa PEG

35%
GA-rate reduction · GATHER1, Y1

12%
Wet AMD safety signal

Dosing: every 25–60 days, IVT injection

WHERE QBR207 ATTACKS THE GAP

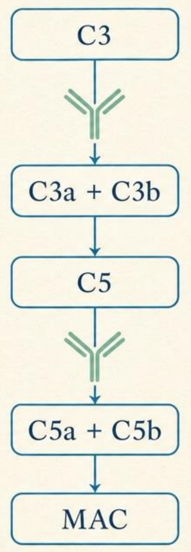
Broader pathway coverage (C3 + C5) · superior efficacy at lower dose · differentiated safety · longer dosing interval.

QUAERITE

First-in-class anti-C3 / anti-C5 bispecific antibody.

Multi-point complement inhibition — broader than SYFOVRE (C3-only) and IZERVAY (C5-only) — tailored to GA pathophysiology.

MECHANISM OF ACTION



QBR207 (Y-shape) blocks both C3 cleavage and C5 cleavage — halts cascade at two checkpoints.

COMPARATIVE COVERAGE

	C3A	C3B	C5A	C5B	PRI-C5
QBR207	✓	✓	✓	✓	✓
IZERVAY	—	—	✓	✓	—
SYFOVRE	✓	✓	*ind*	*ind*	*ind*

✓ direct inhibition · — no effect · ind indirect

QBR207 TAKEAWAY

Broader complement blockade than any approved GA therapy — biparatopic anti-C5 + direct anti-C3.

Better complement inhibition. Restored RPE function.

QBR207 outperforms SYFOVRE and IZERVAY across alternative and classical pathway assays and reverses RPE dysfunction in disease models.

ALTERNATIVE PATHWAY

6×

lower IC50 vs. SYFOVRE

QBR207 blocks alternative complement pathway with 6× greater potency.

CLASSICAL PATHWAY

23×

lower IC50 vs. SYFOVRE

23× more potent on classical complement pathway.

MACROPHAGE MODULATION

>95%

IL-6 cytokine inhibition

Suppresses M1 polarization & inflammatory cytokine release in vitro.

RPE FUNCTIONAL RESCUE

QBR207 restores POS phagocytosis in oxidative-stress-damaged RPE cells.

Translational evidence that complement blockade can preserve photoreceptor function — the precise mechanism that drives GA visual decline.

QUAERITE

A first-in-class complement bispecific built for GA.

Broader mechanism, deeper tissue exposure and projected 2–4 month dosing intervals — re-defining what complement therapy can deliver.

01

Broader mechanism

C3 + C5 + biparatopic coverage — one molecule, multiple checkpoints.

02

Deeper tissue PK

Retina-choroid AUC ~30% of vitreous AUC; T½ ~215 hr in rabbit VH.

03

Longer dosing

Projected Q2–Q4-month dosing vs. monthly SYFOVRE/IZERVAY.

04

Functional rescue

Preserves RPE phagocytosis + inhibits M1 macrophage activation.

MARKET IMPLICATION › **Best-in-class share opportunity in a fast-growing GA market projected to exceed $3B by 2030.**

QUAERITE

LEAD ASSET · FIRST-IN-CLASS

First-in-class triple-targeted VEGF / IL-6 / ANG-2 therapy.

Designed for diabetic macular edema and inflammatory retinopathies — combining clinically validated VEGF and ANG-2 inhibition with potent IL-6 blockade for patients beyond anti-VEGF reach.

DME is a multi-cytokine disease — anti-VEGF addresses only one driver.

Hyperglycemia activates microglia and infiltrating immune cells that release IL-6, ANG-2 and VEGF — together breaking the blood-retina barrier.

PATHOPHYSIOLOGY DRIVERS

Hyperglycemia → microglia activation — AGE accumulation and oxidative stress trigger retinal microglia and Müller cells.

Cytokine storm: IL-6, ANG-2, VEGF — activated cells release pro-inflammatory and pro-angiogenic mediators in concert.

Tight junction disruption — VEGF + IL-6 disrupt ZO-1 and claudin proteins → blood-retina barrier breakdown.

Persistent fluid leakage — macular edema persists despite anti-VEGF; multi-cytokine biology is the missing piece.

DME · SUB-OPTIMAL RESPONSE

30–40%

Of DME patients are sub-optimal anti-VEGF responders.

AT 6 MONTHS

~50%

Show incomplete response; many discontinue.

TOP UNMET NEED

#1

Durability identified by ophthalmologists as top unmet need.

WORKING-AGE ADOPTION

Q20–24w

Minimum dosing interval needed for working-age patient adoption.

Sources: Semin Immunopathol. 2019;41(4):427-445 · Eye (Lond). 2024;38(9):1755-1761 · J Ophthalmol. 2014;4:24-30.

The only triple-target with validated VEGF & ANG-2 + potent IL-6 blockade.

Superior potency over standards of care and best-in-class tight-junction rescue — designed for diabetic and inflammatory retinopathies.

VS. STANDARDS OF CARE

	AFLIBERCEPT	FARICIMAB	RD-1
VEGF-A arm	≈ comparable	~2× more potent	comparable
ANG-2 arm	n/a	~80× more potent	comparable
IL-6 cis-signal	n/a	n/a	~14× more potent
IL-6 trans-sig.	n/a	n/a	~87× more potent

RD-1: anti-VEGF/ANG2/IL6R competitor benchmark.

TRIPLE-TARGET ARCHITECTURE

Integrates anti-VEGF, anti-ANG-2 and anti-IL-6 in a single molecule.

TIGHT-JUNCTION RESCUE

Best-in-class restoration of ZO-1 vs. mono- and dual-target controls.

DURABILITY ADVANTAGE

Outperforms competitors in laser-induced CNV in mice over time.

DE-RISKED BY CLINICAL IL-6 DATA

Aflibercept-resistant DME patients have elevated IL-6 — biomarker-supported.

210

LEAD ASSET · NOVEL MECHANISM

FZD4 / LRP5 targeted Norrin-mimetic.

A tetravalent bispecific that activates the Wnt / β-catenin pathway to restore the blood-retina barrier and re-perfuse damaged retinal vasculature — first-in-class biology with clinical POC in DME.

Activate Wnt / β-catenin to rebuild the blood-retina barrier.

QBR210 mimics endogenous Norrin signaling — a 2:2 FZD4/LRP5 cluster — to restore endothelial tight junctions and re-perfuse ischemic retina.

PATHWAY BIOLOGY

Norrin → FZD4/LRP5 → β-catenin → vascular homeostasis

Tetravalent FZD4/LRP5 engagement rebuilds endothelial tight junctions (claudin-5, ZO-1) and re-perfuses ischemic retinal tissue — independent of VEGF.

CLINICAL POC · AMARONE TRIAL, RESTORET

Wnt-pathway activation works in DME.

+11.2
ETDRS letters gained

−143μm
Retinal thickness reduction

VS. COMPETITION · QBR210 VS. SZN413 + RESTORET

- IN VITRO Higher Wnt/β-catenin activation than SZN413 + Restoret.
- IN VIVO Better suppression of VEGF-induced retinal leakage vs. SZN413 (rabbit).
- VEGF-INDEP. Usable as monotherapy or in combination with anti-VEGF.

A novel-MoA pillar for retinal-ischemia and BRB-restoration franchises.

QBR210 opens an entire second therapeutic lane — complementary to anti-VEGF, supported by external clinical POC.

01

Novel mechanism

Wnt pathway activation — independent of VEGF, complement and IL-6 biology.

02

Best-in-class design

Tetravalent FZD4/LRP5 architecture out-performs Norrin and rival bispecifics in vitro.

03

Clinically de-risked

Restoret (MK-3000) AMARONE Phase 1 in DME validates the Wnt approach (+11.2 ETDRS letters).

04

Broad indication potential

Applicable to DME, DR, DMI, wAMD and pediatric familial exudative vitreoretinopathy.

IMPLICATION › **QBR210 is the foundation for a Wnt-pathway franchise across diabetic and ischemic retinal indications.**

/ THE PLAN

A clear path to clinical value inflection — and beyond.

Roadmap, milestones and partnership pathways that translate our science into category-defining medicines.

Development roadmap & key value-inflection milestones.

Six programs progressing on coordinated timelines — multiple value-creating events across 2025–2027.

2025
Q1 – Q4

QBR202 IND-Enabling
QBR207 PCC nomination
QBR210 Discovery + PCC

2026
Q1 – Q4

QBR202 Phase 1 initiation
QBR207 IND-Enabling
QBR209 IND-Enabling
QBR210 IND-Enabling

2027
Q1 – Q4

QBR202 Phase 1 readout
QBR207 Phase 1 initiation
QBR210 Phase 1 initiation

2028+
THROUGHOUT

QBR202 Phase 2 · POC
QBR207 Phase 1 readout
QBR209 Phase 1 initiation
Pipeline expansion 211 / 212

MULTIPLE BD TOUCHPOINTS

Data readouts in 2026/2027 create natural partnering windows for each asset.

QUAERITE

Where value is created.

Each program offers distinct value-creation events; together they form a stacked, asymmetric portfolio.

QBR202

WAMD · DME · DR · RVO

BIC anti-VEGF / ANG-2

INFLECTION POINTS

Phase 1 readout 2027

Phase 2 POC 2028

Best-in-class label opp.

QBR207

GEOGRAPHIC ATROPHY

FIC anti-C3 / C5 bispecific

INFLECTION POINTS

Phase 1 initiation 2027

Phase 1 readout 2028

First mover in broader complement

QBR209

DME · DR · UME

FIC anti-VEGF / IL-6 / ANG-2

INFLECTION POINTS

IND-Enabling 2026–2027

Phase 1 initiation 2028

Anti-VEGF non-responder niche

QBR210

DME · DR · DMI

Novel Norrin-mimetic

INFLECTION POINTS

IND-Enabling 2026

Phase 1 initiation 2027

Clinically de-risked MoA

We are open. Asset, platform or geography — let's talk.

Quaerite is structured to support multiple partnership formats. Each can be tailored to a partner's capabilities and pipeline gaps.

A Asset out-licensing

License individual programs (e.g., ex-China rights to QBR202 or QBR207) to partners with retinal franchise infrastructure.

OPTIMIZED FOR: PHARMA WITH ESTABLISHED RETINAL COMMERCIAL FOOTPRINT

B Co-development partnerships

Jointly fund and execute clinical development for one or more assets, with shared global rights and milestones.

OPTIMIZED FOR: MID-CAP BIOTECH SEEKING PIPELINE DEPTH

C Platform access · OcuHLE-X®

License OcuHLE-X® to engineer partner-selected antibodies into long-acting ocular biologics.

OPTIMIZED FOR: COMPANIES NEEDING PK EXTENSION

D Geographic carve-outs

Partner regional rights (e.g. Japan, EU, MENA) on lead assets with regional commercial partners.

OPTIMIZED FOR: REGION-SPECIFIC COMMERCIAL LEADERS

/ THE TEAM

Tsinghua-rooted, globally seasoned, retina-obsessed.

A founding team that combines deep scientific authority with operating discipline — and an advisory bench that ranks among the field's leading voices.

Founders & senior management.

World-class scientific authority paired with global operating experience.

FQ

Dr. Feng Qian
FOUNDER · SAB CHAIR

Bayer Endowed Professor & Dean, School of Pharmaceutical Sciences, Tsinghua University
20+ yrs at top research institutes & industry
Globally recognized formulation & drug-delivery scientist
Former Principal Scientist, BMS · US-China drug-development expert

XZ

Mr. Xiaoqing Zhu
CO-FOUNDER · COO

10+ yrs senior management & operations in life-science MNCs
Former Head, Biological Agriculture BU, Novozymes China
Director, China Strategy & Business Development

LC

Dr. Li Chen
CTO

20+ yrs biologics development experience
Expertise: protein drug design, upstream/downstream process, pre-clinical & early clinical
Developed biologics across all stages — including marketed Zamerovimab/Mazorelvimab

LL

Mr. Lei Liu
VP, STRATEGY & BD

BD experience at NASDAQ-listed biotech
Senior positions in IVD industry and healthcare venture capital firms

Scientific & industrial advisors.

Anchored by some of the world's leading voices in retinal disease and ocular drug development.

TY

Tien Yin Wong, MPH PhD

FOUNDING DEAN, TSINGHUA MEDICINE

Leading global expert in retinal diseases

Academician, National Academy of Sciences of Singapore

Foreign Academician, National Academy of Medicine, USA

BX

Bailong Xiao, PhD

PROFESSOR, TSINGHUA PHARM. SCIENCES

Principal Investigator, IDG/McGovern Institute for Brain Research

New Cornerstone Investigator

Global leader in PIEZO ion-channel research · key contributor to the 2021 Nobel Prize in Physiology or Medicine

HH

Henry Hsu, MD

CEO, ALLYSTA PHARMACEUTICALS

Former Founder & CEO, Altheos · Former CMO at Plexxikon, Molecular Partners, CoMentis, CoTherix

25+ years of drug development experience

Focus: ophthalmology, oncology and liver diseases

LET'S BEGIN A CONVERSATION

Let's seek cures.
Together.

Quaerite is open to assets, platform and geographic partnership conversations.

CONTACT

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